[HOSPITAL NAME]

[Full Address, City, State — PIN Code]
Phone: _____ | Emergency: _____ | Email: _____
NABH Accreditation No.: _____ | Reg. No.: _____

Form No: SC-OT-001
Version: 3.1 | Effective: April 2025
Department: _____
IPD / OPD No.: _____

INFORMED CONSENT FOR SURGICAL PROCEDURE, ANAESTHESIA AND RELATED MEDICAL INTERVENTIONS

(To be completed in full before the patient is taken to the Operation Theatre)

A. PATIENT IDENTIFICATION AND ADMISSION DETAILS

Full Name of Patient: _____

Age: _____ **Gender:** ☐ Male ☐ Female ☐ Other _____

Date of Birth: _____ **Blood Group:** _____

Address of Patient: _____

IPD Registration No.: _____ **Ward / Bed No.:** _____

Date of Admission: _____ **Date of Proposed Surgery:** _____

Consultant Surgeon (Name & Qualification): _____

Anaesthesiologist (Name & Qualification): _____

Scrub Nurse / OT Coordinator: _____

Resident / Assisting Doctor: _____

B. DIAGNOSIS AND PROPOSED SURGICAL PROCEDURE

Provisional / Confirmed Diagnosis: _____

Name of Proposed Surgical Procedure: _____

Nature of Surgery: _____

☐ Elective ☐ Emergency / Urgent ☐ Exploratory

Side / Laterality (if applicable): ☐ **Left** ☐ **Right** ☐ **Bilateral** ☐ **N/A** _____

Site / Body Part to be Operated: _____

Description of Procedure (to be completed by the Surgeon):

Proposed Date & Time of Surgery: _____

Estimated Duration of Surgery: _____

Type of Anaesthesia: ☐ **General** ☐ **Spinal** ☐ **Epidural** ☐ **Regional** ☐ **Local** ☐ **MAC** _____

C. PRE-OPERATIVE CLINICAL INFORMATION

C.1 Relevant Medical History

Does the patient have any known drug allergies?	☐ YES	☐ NO

If YES, specify drugs / substances: _____

Does the patient have any known allergies to latex, adhesives, or surgical materials?	☐ YES	☐ NO

If YES, specify: _____

Does the patient have any history of adverse reaction to anaesthesia?	☐ YES	☐ NO

If YES, describe: _____

Has the patient undergone any previous surgical procedures?	☐ YES	☐ NO

If YES, specify procedure(s) and year(s): _____

Is the patient currently on any regular medication, anticoagulants, or blood thinners?	☐ YES	☐ NO

If YES, list medications: _____

C.2 Current Clinical Status

Blood Pressure: _____ **Pulse Rate:** _____

SpO2 (%): _____ **Respiratory Rate:** _____

Weight (kg): _____ **Height (cm):** _____

BMI: _____ **ASA Classification:** _____

Is the patient fasting (NPO status confirmed)?	☐ YES	☐ NO

Duration of Fasting (hours): _____

Is the patient pregnant or possibly pregnant? (Female patients only)	☐ YES	☐ NO
Has pre-operative blood investigation been completed and reviewed?	☐ YES	☐ NO
Has pre-operative imaging (X-ray / ECG / Echo / USG / CT / MRI) been reviewed?	☐ YES	☐ NO
Has anaesthetic fitness / pre-anaesthetic check-up (PAC) been completed?	☐ YES	☐ NO
Has consent for blood transfusion been separately obtained if required?	☐ YES	☐ NO

D. NATURE AND SCOPE OF INFORMED CONSENT

STATEMENT OF ATTENDING SURGEON

I, the undersigned Consultant Surgeon, hereby certify that I have explained the following to the patient and/or their authorised representative in a language and manner they have understood:

1. The nature, purpose, and objectives of the proposed surgical procedure, including the expected benefits of undergoing the surgery.
2. The type of anaesthesia to be administered, its purpose, and general effects on the body during and after surgery.
3. The material risks, complications, and possible adverse events associated with the proposed surgery and anaesthesia, including but not limited to those described in Section E of this form.
4. The available alternative treatment options, including non-surgical management, and the potential consequences of refusing or delaying the procedure.
5. The possibility that, during the course of surgery, the surgeon may encounter unforeseen conditions necessitating a modification of the planned procedure, performance of an additional procedure, or a change in operative approach, and consent is hereby sought for such intra-operative decisions.
6. The expected duration of hospital stay, post-operative care requirements, and approximate recovery timeline.
7. The limitations of medical science and the fact that no guarantee of a specific outcome or result can be given or implied.
8. The use of implants, prosthetics, surgical mesh, hardware, bone grafts, or other medical devices, if applicable to this procedure.
9. The possibility of requiring intra-operative or post-operative blood transfusion and the associated risks.
10. The involvement of resident doctors, medical students, or trainee surgeons in the procedure for educational purposes, subject to the patient's consent as indicated in Section G.

E. RISKS, COMPLICATIONS AND ADVERSE EVENTS

IMPORTANT: This section discloses known risks associated with surgical procedures and anaesthesia. The absence of a specific risk from this list does not imply that such risk does not exist. Your surgeon will discuss risks specific to your condition and procedure.

E.1 General Risks Common to All Surgical Procedures

1. Bleeding / haemorrhage — intra-operative or post-operative, which may necessitate blood transfusion or re-operation.
2. Infection — at the surgical site, in deeper tissues, or systemic (sepsis), which may require antibiotic therapy or additional surgical intervention.
3. Adverse reaction to anaesthetic agents, analgesics, or medications administered during the perioperative period, including anaphylaxis.
4. Deep Vein Thrombosis (DVT) and Pulmonary Embolism (PE) — formation of blood clots in veins, which may travel to the lungs and become life-threatening.
5. Wound dehiscence — breakdown or failure of the surgical wound to heal, which may require wound care, secondary closure, or further surgery.
6. Injury to adjacent structures including blood vessels, nerves, or organs in the vicinity of the operative site.
7. Post-operative nausea, vomiting, pain, and temporary or permanent altered sensation in the operative area.
8. Respiratory complications including pneumonia, atelectasis, or respiratory failure, particularly in patients with pre-existing lung conditions.
9. Cardiovascular events including myocardial infarction (heart attack), arrhythmia, or cardiac arrest.
10. Anaesthesia awareness — rare occurrence of consciousness during general anaesthesia.

11. Death — in rare but documented cases, surgery and anaesthesia may result in death. The risk varies significantly with the nature of the procedure, the patient's overall health, and the urgency of the surgery.

E.2 Risks Specific to Spinal / Epidural / Regional Anaesthesia

1. Headache following spinal or epidural puncture (Post-Dural Puncture Headache — PDPH).
2. Failure of regional block necessitating conversion to general anaesthesia.
3. Nerve injury resulting in temporary or permanent numbness, weakness, or paralysis.
4. Hypotension (low blood pressure) requiring fluid administration and/or vasopressors.
5. Epidural haematoma or abscess — rare but potentially causing permanent neurological deficit.

E.3 Risks Specific to This Procedure (To be completed by Surgeon)

F. CONSENT FOR BLOOD TRANSFUSION AND BLOOD PRODUCTS

Blood transfusion may be required during or after surgery in cases of significant blood loss. The following blood products may be used: Packed Red Blood Cells (PRBC), Fresh Frozen Plasma (FFP), Platelets, Cryoprecipitate, or Whole Blood.

Risks associated with blood transfusion include: transfusion reactions (febrile, haemolytic, allergic), transmission of infectious agents (extremely rare with current screening), volume overload, and transfusion-related acute lung injury (TRALI).

☐ I CONSENT to the administration of blood and/or blood products during or after surgery if deemed medically necessary by the treating team.

☐ I DO NOT CONSENT to blood transfusion. I understand this may increase the risk of complications and mortality. I accept full responsibility for this decision. (If checked, please provide written reason and alternative preference below.)

Reason / Alternative (if not consenting to transfusion): _____

Religious or other objection to blood transfusion:

☐ Jehovah's Witness ☐ Other Religious Grounds ☐ Personal / Ethical Grounds

G. ADDITIONAL CONSENTS AND AUTHORISATIONS

G.1 Tissue / Specimen Disposal

Tissue, organs, or specimens removed during surgery may be sent for pathological examination. Please indicate your preference for disposal of such tissue:

☐ I consent to removed tissue / specimens being used for pathological examination and thereafter disposed of by the hospital in accordance with applicable biomedical waste regulations.

☐ I consent to anonymised tissue / specimens being used for approved medical research or teaching purposes, in addition to pathological examination.

☐ I do NOT consent to use of tissue / specimens for research or teaching. Tissue should be disposed of following pathological examination only.

G.2 Photography, Videography and Documentation

☐ I consent to intra-operative photography and videography of the surgical procedure for medical record purposes only.

☐ I consent to intra-operative photography / videography being used for medical education, teaching, or academic publication (with identity anonymised).

☐ I DO NOT consent to any photography or videography during my procedure.

G.3 Medical Training and Student Involvement

☐ I consent to the presence and supervised participation of resident doctors, post-graduate medical students, and surgical trainees during my procedure, for the purposes of medical education.

☐ I DO NOT consent to the presence of trainees or students during my procedure.

G.4 Implants and Medical Devices

Does this procedure involve the use of any implant, prosthesis, mesh, fixation device, or foreign material?	☐ YES	☐ NO

If YES, specify type and make of implant / device: _____

☐ I consent to the use of the implant / device described above. I have been informed of the material composition, expected lifespan, and associated risks of the implant.

H. ALTERNATIVE TREATMENTS AND CONSEQUENCES OF REFUSAL

H.1 Alternative Treatment Options Discussed

Option 1: _____

Option 2: _____

Option 3: _____

H.2 Consequences of Refusing or Delaying Surgery

I. RIGHT TO WITHDRAW CONSENT

The patient retains the absolute right to withdraw this consent at any time prior to the commencement of anaesthesia induction. Once anaesthesia has been induced and surgery has commenced, withdrawal of consent mid-procedure may not be possible, and the surgical team is authorised to complete procedures necessary for the patient's immediate safety. Withdrawal of consent after surgery has begun will be evaluated by the attending surgeon and anaesthesiologist on a case-by-case basis.

I understand that if I withdraw consent before surgery commences, I will not be subjected to any punitive measure or denial of future medical care. The hospital and its staff shall inform me of the medical consequences of withdrawal to enable an informed decision.

J. PATIENT / GUARDIAN DECLARATION AND CONSENT

I / WE HEREBY DECLARE AND CONFIRM AS FOLLOWS:

(Please read each statement carefully and initial against each item)

- ☐ I have been informed of the nature, purpose, expected benefits, and risks of the proposed surgical procedure and anaesthesia in a language I understand. *Initial* _____
- ☐ I have been given sufficient opportunity to ask questions, and all my questions have been answered to my satisfaction. *Initial* _____
- ☐ I understand that surgery involves inherent risks and that no guarantee of outcome has been given or implied. *Initial* _____
- ☐ I have been informed of the alternative treatment options and the consequences of refusing or delaying surgery. *Initial* _____

☐	I am making this decision voluntarily, without coercion, pressure, or undue influence from any person.	Initial _____
☐	I consent to the performance of the proposed surgery and anaesthesia as described in this form, and to any additional procedures that may become necessary during the course of surgery to address unforeseen findings or to preserve my life or health.	Initial _____
☐	I consent to post-operative care, including administration of medications, intravenous fluids, and other treatments, as deemed necessary by the treating team.	Initial _____
☐	I have disclosed all known allergies, current medications, prior surgical history, and relevant medical conditions to the attending medical team.	Initial _____
☐	I understand that I have the right to withdraw this consent before anaesthesia is administered.	Initial _____
☐	I authorise the hospital and its medical staff to contact my next of kin or emergency contact in case of any complication or unforeseen event arising during or after surgery.	Initial _____

K. CONSENT BY LEGAL GUARDIAN / AUTHORISED REPRESENTATIVE

(To be completed ONLY if the patient is: a minor under 18 years of age / unconscious or sedated / physically unable to sign / assessed as lacking capacity to give informed consent)

Reason patient cannot personally provide consent:

☐ Minor / Under 18 ☐ Unconscious / Sedated ☐ Lacks Capacity ☐ Physical Inability

Full Name of Guardian / Representative: _____

Relationship to Patient: _____

ID Proof Type: _____ **ID Proof Number:** _____

Address of Guardian / Representative: _____

Contact Number: _____

I confirm that I am the legal guardian / authorised representative of the above-named patient, that I have been given all information required to make an informed decision on the patient's behalf, and that I am consenting to this procedure in the patient's best interests.

L. SIGNATURES AND ATTESTATION

L.1 Patient Signature

Signature of Patient _____ *Thumb Impression of Patient (if unable to sign)* _____

Full Name: _____ **Date:** _____

Time of Signing: _____

L.2 Legal Guardian / Representative Signature (if applicable)

Signature of Guardian / Representative _____ *Date & Time:* _____

Full Name: _____ **Relationship:** _____

L.3 Witness Signatures

Signature of Witness 1 _____ *Signature of Witness 2* _____

Name (Witness 1): _____ **Name (Witness 2):** _____

Designation: _____ **Designation:** _____

L.4 Attending Surgeon Attestation

I hereby certify that I have explained the procedure, its risks, benefits, and alternatives to the patient and/or their guardian in a language they understood, that the patient/guardian had the opportunity to ask questions, and that the consent documented on this form was given freely and voluntarily, without coercion.

Signature of Consultant Surgeon _____ *Date & Time:* _____

Name: _____ **Registration No. (MCI / NMC):** _____

L.5 Anaesthesiologist Attestation

I hereby certify that I have conducted the Pre-Anaesthetic Check-up (PAC) and explained the type of anaesthesia, its risks, and alternatives to the patient and/or their guardian.

_____ _____

Signature of Anaesthesiologist *Date & Time:*

Name: _____ **Registration No. (MCI / NMC):** _____

M. INTERPRETER DECLARATION (IF APPLICABLE)

(To be completed if an interpreter was used during explanation of this consent form)

I hereby certify that I accurately and completely interpreted the contents of this consent form and the explanations provided by the surgeon and anaesthesiologist to the patient and/or their guardian from English into:

Language interpreted: _____

_____ _____

Signature of Interpreter *Date & Time:*

Name: _____ **Contact No.:** _____

N. REVOCATION / WITHDRAWAL OF CONSENT

(To be completed ONLY if the patient wishes to withdraw consent before commencement of anaesthesia)

I, _____, the patient / legal guardian named in this form, hereby withdraw the consent given under this form.

Reason for withdrawal (optional): _____

I understand the medical consequences of withdrawing consent as explained to me by the attending surgeon:

_____ _____

Signature of Patient / Guardian *Signature of Surgeon (acknowledging withdrawal)*

Date: _____ **Time:** _____